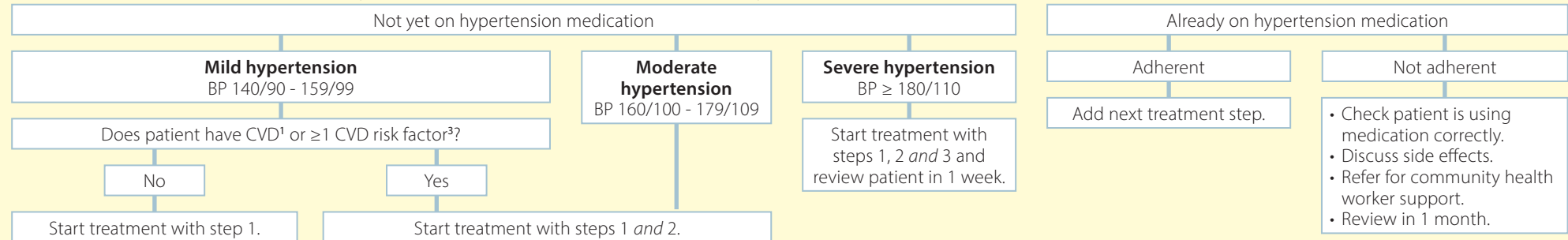


Treat the patient with hypertension

- Give **influenza vaccine** 0.5mL IM yearly. Check that patient is up to date with his/her COVID-19 vaccine.
- If **known CVD**¹:
 - Give **simvastatin**² 40mg at night. If on amlodipine, give instead **simvastatin**² 20mg at night. No dose adjustment needed for rosuvastatin, pravastatin, atorvastatin. If patient develops muscle pain/cramps, reduce dose to 10mg at night. Avoid if pregnant or liver disease.
 - Give **aspirin** 150mg daily. Avoid if peptic ulcer, dyspepsia, kidney or liver disease.
- If **no known CVD**¹: if CVD risk > 20%, give **simvastatin**² 10mg at night. Avoid if pregnant or liver disease.
- If BP is **controlled**, continue current treatment step and review 6 monthly.
- If BP is **not controlled**, decide treatment for hypertension using algorithm and table below. If already on step 7, refer instead.



Step	Medication	Note
1	Address modifiable CVD risk factors.	Manage CVD risk ↗ 129. If BP not controlled after 3 months, add step 2.
2	Add hydrochlorothiazide (HCTZ) 12.5mg daily.	• Avoid if pregnant, personal/family history of skin cancer, gout, severe liver disease or eGFR < 30. • If diabetes or heart failure, start enalapril 10mg daily instead of HCTZ. Then if needed, add HCTZ as next step once on maximum dose of enalapril.
3	Add enalapril 10mg at night.	• Avoid if pregnant, eGFR < 30 or potassium ≥ 5.0. • Advise patient to stop enalapril immediately if swelling of tongue/lips/face develops, angioedema likely ↗ 32.
4	Increase enalapril to 20mg at night.	
5	Add amlodipine 5mg at night.	Avoid if untreated heart failure. If on simvastatin, reduce simvastatin dose to 20mg at night. No dose adjustment needed for rosuvastatin, pravastatin, atorvastatin.
6	Increase amlodipine to 10mg at night.	
7	Add spironolactone 25mg daily and increase HCTZ to 25mg daily.	Only use spironolactone if potassium can be monitored. Avoid spironolactone if pregnant or eGFR < 30.

- Review the patient monthly until BP controlled. Once controlled, review 6 monthly.
- If BP not controlled after 1 month on step 7, refer.

¹Cardiovascular disease (CVD) includes ischaemic heart disease, peripheral vascular disease and stroke/TIA. ²If on lopinavir/ritonavir or atazanavir/ritonavir, avoid simvastatin, give instead **atorvastatin** 10mg at night. ³CVD risk factors include age > 55 (man) or > 65 (woman), diabetes, smoker, waist circumference > 80cm (woman) or > 94cm (man).